

Do Gender and Racial Differences Between Patient and Therapist Affect Therapeutic Alliance and Treatment Retention in Adolescents?

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Parents, referral sources, and even therapists wonder whether the gender and racial match between therapists and patients contributes to poorer alliances and treatment dropout. Six hundred adolescent substance abusers and their therapists from a large randomized clinical trial were grouped according to matches and mismatches on both gender and race, and alliance ratings were collected from both patients and therapists. Results revealed that gender-matched dyads reported higher alliances and were more likely to complete treatment. Racial matching predicted greater retention but not patient-rated alliance. However, therapists in mismatched dyads rated significantly lower alliances. Results suggest that, although multicultural training remains critical, training emphasis should also be placed on understanding how gender and racial differences affect therapeutic processes.

Keywords: gender matching, racial matching, therapeutic alliance, adolescent therapy

One often wonders whether demographic differences or similarities between patient and therapist, such as gender and race, affect therapy processes. Clinical lore drives many assumptions that these elements, with interpersonal consequences, affect therapy beyond the actual intervention. For example, there is a common belief that boys will demonstrate greater improvement with male therapists and girls with female therapists. This is particularly true when working with children and adolescents, as therapists

seek to address problematic behavior while attending to specific developmental needs. Similarly, there is an assumption that a patient–therapist match on race will also maximize treatment retention and outcome. Unfortunately, there is a paucity of data on child/adolescent psychotherapy to shed light on these questions. This article examined the effect of gender and racial matching on two key process variables: therapeutic alliance and treatment retention.

The concept of matching patients and therapists on the basis of gender or race has been explored in adult psychotherapy. In an early review of the psychotherapy outcome literature, Luborsky, Auerbach, Chandler, Cohen, and Bachrach (1971) concluded that a patient–therapist match on gender results in better outcomes. Much of the theory on gender matching comes from the social psychology literature, which suggests that people tend to identify with individuals similar to themselves (Festinger, 1954), and from historical beliefs that matching would contribute to positive therapeutic change (e.g., Fabrikant, 1974). Bem's (1981) gender schema theory may also explain patient preference for therapists similar to themselves. She asserted that individuals rely on a gender schema to process information according to cultural definitions of maleness and femaleness. Thus, patients and therapists of the same sex would tend to view the world through the same gender lens, presumably promoting similar perspectives over problematic issues. Furthermore, object relations theories have been used to understand the role of the therapist as a transitional object to help children manage loss (e.g., absent parent) by temporarily replacing the lost object and helping to resolve destructive experiences (Shainess, 1983). For example, a boy with an absent father may experience greater therapeutic gains with a male clinician.

Some research findings have emerged concerning gender matching. Generally, female patients tend to prefer female therapists (Fowler, Wagner, Iachini, & Johnson, 1992; Jones, Krupnick,

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& Kerig, 1987). In addition, female therapists rate their patients as more satisfied with therapy than male therapists (Jones & Zoppel, 1982) and tend to form stronger self-reported therapeutic alliances with their patients (Dolinsky, Vaughn, Luber, Mellman, & Roose, 1998; Jones & Zoppel, 1982).

Despite the general preferences cited above, research fails to demonstrate that gender matching leads to either improved outcomes (Cottone, Drucker, & Javier, 2002; Nelson, 1993; Zlotnick, Elkin, & Shea, 1998) or decreased dropout (Sterling, Gottheil, Weinstein, & Serota, 1998). Even with respect to sexually abused girls (aged 7–15), Fowler and Wagner (1993) were able to demonstrate that despite a pretreatment preference for a female therapist, those assigned male therapists were no less comfortable with their therapist upon treatment completion than their counterparts treated by female therapists. Furthermore, DeHeer, Wampold, and Freund (1992) found that undergraduates not currently in therapy preferred therapists on the basis of effectiveness rather than gender, again mitigating the relative importance of gender matching.

The importance of patient–therapist match on race is also controversial. Many studies clearly demonstrate that low-income minority individuals are less likely to seek and complete mental health treatment (Agosti, Nunes, & O'cepek-Welikson, 1996; Rebach, 1992). Not surprisingly, research demonstrates that African American patients consistently prefer to see African American therapists rather than Caucasian therapists (Atkinson, 1983; Helms & Carter, 1991; Thompson, Bazile, & Akbar, 2004). As with gender, social psychology suggests that racial preferences may be attributable to the belief that similar appearances signify similar attitudes (Atkinson, 1983). In order to address these problems, clinicians and researchers have established guidelines for using culturally competent treatment practices, as recently delineated by Stuart (2004). Of course one strategy to ensure cultural competence is to simply match patients and therapists on race whenever possible (Campbell & Alexander, 2002; Sue, Fujino, Hu, Takeuchi, & Zane, 1991). By doing so, perhaps a greater initial trust between patient and therapist, resulting from a common cultural identity, will render similar beliefs with respect to mental health (Thompson et al., 2004; Wong, Kim, Zane, Kim, & Huang, 2003). However, even given a racial match, some participants in Thompson et al.'s (2004) study, in which qualitative methods were used for understanding the underutilization of mental health services in the African American community, feared that African American psychologists would be elitist and therefore “too far removed from the culture and as insensitive as therapists who were not African American” (p. 24). Nevertheless, although racial matching has not shown improved treatment outcomes (Maramba & Hall, 2002), several studies have found racial matching leads to greater service utilization and retention (Maramba & Hall, 2002; Sue et al., 1991; Yeh, Eastman, & Cheung, 1994).

No studies have examined whether gender or racial match affect the formation of early therapeutic alliance. As Greenson (1967) proclaimed years ago, therapeutic alliance may be the most essential element in effective therapy. The strength and quality of the interpersonal relationship between patient and therapist plays an important role in the curative process (e.g., Sullivan, 1940). A recent meta-analysis of therapeutic alliance and outcomes in child and adolescent psychotherapy (Shirk & Karver, 2003) supports this long-standing belief by concluding that positive relationships (i.e., strong therapeutic alliance) between patient and therapist

result in better treatment outcomes. Understanding how match affects this important therapeutic process may help illuminate the role that match plays in treatment effectiveness.

Unfortunately, almost no data exist on how matching affects treatment with adolescents, let alone those in treatment for substance abuse. Therefore, the present study explored the effects of gender and racial matching on two key treatment indicators, the therapeutic alliance and retention, in a sample of substance-abusing adolescents. Engagement and retention have been found to be important predictors of successful treatment within substance-abusing populations (Hopfer, Khuri, Crowley, & Hooks, 2002; Simpson, Joe, & Rowan-Szal, 1997), although little is known about the effects with adolescents. Additionally, we investigated differences between patient-rated alliance and therapist-rated alliance to discern whether matched and mismatched therapists rate alliance similarly to their patients. On the basis of previous research, we have several expected findings: (a) Gender-matched dyads will report stronger alliances, specifically female patients matched with female therapists; (b) gender matching will not predict retention; and (c) racial matching will predict both alliance and retention.

Early Alliance and Treatment Retention

The sample for this study included participants from the Cannabis Youth Treatment (CYT) project. The project was funded by the Center for Substance Abuse Treatment to evaluate five outpatient models of treatment for adolescents with cannabis abuse or dependence diagnoses at four different sites. All treatment models were manual guided and consisted of individual, group, and family models ranging from 5 to 21 sessions in length (Dennis et al., 2002, presents a detailed description of the research design). At the time point in treatment where data were collected for the present study, two thirds of patients completed two sessions of individual Motivational Enhancement Therapy, one sixth completed two sessions of an individual behavioral therapy, and one sixth completed two sessions of Multidimensional Family Therapy (see Diamond et al., 2002).¹

Adolescents were excluded from the study if they met any of the following criteria: (a) had an acute medical or psychological condition that required immediate treatment or was likely to prohibit full participation in treatment; (b) appeared to have insufficient mental capacity to understand the consent and treatment process; (c) had a history of violent, severe conduct, or predatory behaviors; or (d) lacked a parent or other caretaker willing to consent for treatment and participate in the research assessments and possibly treatment. Of the 1,244 adolescents screened through the intake streams, 44% were ineligible, with over 20% being too severe for outpatient treatment. Of the 702 who were eligible, 600 (85%) agreed to participate, or 48% of those screened. Sample characteristics included 85% over age 15 (average age = 15.7 years), 61% Caucasian, 32% African American, and 19% girls. Of

¹ All three early treatment phases included specific strategies to engage adolescents, and, therefore, differences among the various treatment models are not viewed as key aspects to this article. The reader is referred to Diamond et al. (2002) for a comprehensive description of each treatment model. Also, the treatment manuals may be downloaded at <http://www.chestnut.org/LI/cyt/index.html>

Table 1
Gender Differences in Patient and Therapist Ratings of the Therapeutic Alliance

Category	WAIP (N = 352)				WAIT (N = 347)			
	<i>n</i>	<i>M</i> or <i>M</i> diff.	<i>SD</i>	<i>p</i>	<i>n</i>	<i>M</i> or <i>M</i> diff.	<i>SD</i>	<i>p</i>
Same gender	147	73.91	10.24		151	63.62	10.56	
Different gender	205	70.06	12.58		196	61.28	12.84	
Gender comparison		3.85		.00		2.34		.07
1. Male PT–Male TH	101	73.08	11.26		110	62.43	10.33	
2. Male PT–Female TH	183	69.51	12.62		175	61.50	12.89	
3. Female PT–Male TH	22	74.59	11.52		21	59.48	12.51	
4. Female PT–Female TH	46	75.74	7.31		41	66.83	10.61	
1. vs. 2.		3.57		.06		0.93		.92
1. vs. 3.		–1.51		.94		2.95		.72
1. vs. 4.		–2.66		.57		–4.40		.18
2. vs. 3.		–5.08		.21		2.02		.88
2. vs. 4.		–6.23		.01		–5.33		.05
3. vs. 4.		–1.15		.98		–7.35		.10

Note. *M* diff. = mean difference and is based on comparisons between the specific gender categories. The Working Alliance Inventory is based on a 7-point Likert scale. WAIP = Patient version of the Working Alliance Inventory; WAIT = Therapist version of the Working Alliance Inventory; PT = patient; TH = therapist.

the participants, 62% were involved with juvenile justice, 52% came from single-parent homes, 85% first used drugs before the age of 15, and 75% had no prior treatment experience. On the basis of a combined parent and adolescent report, 82% of the sample had co-occurring mental health problems, including conduct disorder (74%), attention-deficit/hyperactivity disorder (78%), depression (38%), and traumatic distress (14%), and 72% had multiple co-occurring mental health problems (Diamond, Panichelli-Mindel, Shera, Tims, & Ungemack, in press).

Fourteen therapists were recruited for the CYT project from both within and outside the provider agencies. Therapists had a mean age of 39.3 years, and nine were women. Seven therapists were Caucasian women, four were Caucasian men, one was an African American woman, one was an African American man, and one was Latina. Eight therapists had a master's degree, three had a bachelor's degree, and three had a doctoral degree. Therapists averaged 5.7 years of experience working with adolescents (*SD* = 5.4). All therapists read the treatment manuals and were rigorously trained in their respective modalities with the model developer. In order to gauge adherence to the intervention, two audio recorded or videotaped sessions were completed every month for each therapist, which were subsequently monitored by supervisors using treatment-specific adherence measures.

For the purpose of the present investigation, patients and therapists were categorized on the basis of gender and racial match. This was not, however, a specified intervention strategy in the CYT project. Two broad categories were created for gender: same gender and different gender. Four additional groups were created for the purposes of examining specific patient–therapist gender clusters: (a) male patient–male therapist, (b) male patient–female therapist, (c) female patient–male therapist, and (d) female patient–female therapist. Similarly, race was dichotomized into two broad categories: same race and different race. Here, for example, an African American patient being treated by a Latina therapist would be defined as *different race*, as would an African American patient being treated by a Caucasian therapist. Unfortu-

nately, there was not enough power to consider patient–therapist analyses across all specific ethnic or cultural categories. However, in an effort to delineate differences on the basis of multiple racial-matching dyads, the following broader racial patient–therapist clusters were created: (a) Caucasian patient–Caucasian therapist, (b) Caucasian patient–minority therapist, (c) minority patient–Caucasian therapist, and (d) minority patient–minority therapist. *Caucasian* is defined as patients and therapists who described themselves as Caucasian, whereas *minority* is defined as patients and therapists who described themselves using any other racial category. For patients, 82% of the minority category identified themselves as African American. For therapists, recall that two African Americans and one Latina composed the minority category.² Thus, it is important to note that although a Latina therapist working with an African American adolescent falls into the “different race” category, when we break it down further into specific patient–therapist clusters, this dyad would be considered in Group 4, minority patient–minority therapist. Sample sizes for each matched category varied on the basis of the availability of the dependent measures in the data and are indicated in Tables 1, 2, and 3.

² Categories were divided into Caucasian and minority in an effort to offer a more complete analysis. Specifically, we wanted to tease apart the effects of minority patients working with Caucasian therapists and vice versa. Our original categories of “same race” and “different race” do not allow for such analyses, as both Caucasian patient–minority therapist and minority patient–Caucasian therapist fall under the same broad category of “different race.” Furthermore, categorizing our single Latina therapist was methodologically complex. Our Latina therapist was of South American descent. After discussing her own experiences of acculturation, social and ethnic stigma, and so forth with her, we felt it was most appropriate to include her in the minority category, as her experiences were most analogous to the other minority participants in the study. Finally, moving this therapist into the Caucasian category and redefining the categories did not statistically impact the results.

Table 2
Racial Differences in Patient and Therapist Ratings of the Therapeutic Alliance

Category	WAIP (N = 334)				WAIT (N = 327)			
	<i>n</i>	<i>M</i> or <i>M</i> diff.	<i>SD</i>	<i>p</i>	<i>n</i>	<i>M</i> or <i>M</i> diff.	<i>SD</i>	<i>p</i>
Same race	222	72.07	10.99		211	64.09	9.78	
Different race	112	70.48	13.53		116	59.23	14.60	
Racial comparison		1.59		.25		4.86		.00
1. Caucasian PT–Caucasian TH	201	71.95	11.02		190	64.52	9.67	
2. Caucasian PT–Minority TH	13	73.00	10.54		23	53.83	15.03	
3. Minority PT–Caucasian TH	83	69.45	13.86		76	58.75	14.07	
4. Minority PT–Minority TH	37	73.46	12.10		38	64.00	11.89	
1. vs. 2.		–1.05		.99		10.69		.00
1. vs. 3.		2.50		.37		5.77		.00
1. vs. 4.		–1.51		.89		0.52		.99
2. vs. 3.		3.55		.75		–4.92		.28
2. vs. 4.		–0.46		.99		–10.17		.01
3. vs. 4.		–4.01		.32		–5.25		.10

Note. The Working Alliance Inventory is based on a 7-point Likert scale. WAIP = Patient version of the Working Alliance Inventory; WAIT = Therapist version of the Working Alliance Inventory; *M* diff. = mean difference; PT = patient; TH = therapist.

The Working Alliance Inventory (WAI; Horvath & Greenberg, 1989) is based on Bordin's (1979) theoretical work and is designed to assess three aspects of alliance: tasks, goals, and bonds. To lessen the burden on the participants and to increase the likelihood of compliance, the 12-item version of this instrument (Tracey & Kokotovic, 1989) was used instead of the original 36-item version.

Alpha for the total scale was .95, and principal-components analysis resulted in a one-factor solution accounting for 56% of the variance. Adolescents used the WAI to report on their view of the therapeutic alliance (patient version = WAIP). Therapists used the WAI to report on how they believed the patients would rate the therapeutic alliance (therapist version = WAIT). The mean WAIP

Table 3
Gender and Racial Differences in Treatment Retention

Category	Completed two thirds of treatment				
	<i>n</i>	<i>F</i>	%	χ^2	<i>p</i>
Same gender	242	187	77		
Different gender	329	218	66		
Gender comparison				8.20	.01
1. Male PT–Male TH	177	136	77		
2. Male PT–Female TH	294	191	65		
3. Female PT–Male TH	35	27	77		
4. Female PT–Female TH	65	51	78		
1. vs. 2.				7.33	.01
1. vs. 3.				0.01	.97
1. vs. 4.				0.07	.79
2. vs. 3.				2.07	.15
2. vs. 4.				4.41	.04
3. vs. 4.				0.02	.88
Same race	354	281	79		
Different race	188	104	55		
Racial comparison				34.54	.00
1. Caucasian PT–Caucasian TH	311	247	79		
2. Caucasian PT–Minority TH	41	27	66		
3. Minority PT–Caucasian TH	122	59	48		
4. Minority PT–Minority TH	68	52	76		
1. vs. 2.				3.87	.05
1. vs. 3.				40.79	.00
1. vs. 4.				0.29	.59
2. vs. 3.				3.77	.05
2. vs. 4.				1.45	.23
3. vs. 4.				14.20	.00

Note. PT = patient; TH = therapist.

score for the sample was 71.9 ($SD = 11.9$); the mean WAIT score for the sample was 63.2 ($SD = 11.6$).

In these analyses, all adolescents completed the alliance measure at their second or third therapy sessions ($n = 352$ for WAIP, $n = 347$ for WAIT). Because of limited staff support and logistical constraints (e.g., research therapists working alone in a satellite clinic), the therapists issued the alliance measure to the adolescent. Adolescents were informed about the nature of the instrument and promised confidentiality of their answers from their therapists. They filled out the measure on their own, put it in an envelope, sealed it, and left it in a box in the waiting room. Therapists also completed the measure at this time.

It should be noted that both therapist years of experience and degree were significantly related to alliance in this sample. Patients reported higher alliances with therapists with less experience ($r = -.15$, $p = .001$), and these therapists also reported higher alliances with their patients ($r = -.15$, $p = .001$). Although no significant difference emerged for patient-rated alliance on the basis of therapist degree, $F(3, 356) = 2.21$, ns , there was a main effect for therapist-rated alliance on degree, $F(3, 349) = 15.14$, $p < .001$, with bachelor-level therapists reporting the highest alliances. Given the influence of these therapist factors on alliance, we have controlled for them in our analyses.

Treatment retention was measured as face-to-face office-based contacts, and because the interventions varied in their length, *retention* was defined as attending two thirds of the intended treatment sessions. Although studies on the dose-effect relationship has generated contradictory findings, it is generally accepted that providing an individual at least two thirds of the sessions intended for the specific intervention constitute an adequate dose (Dennis et al., 2004). For this reason, we have considered attendance of two thirds of the intended sessions satisfactory treatment completion. For our comparisons, 95% of patients ($n = 571$) in the gender-match analyses had available data on retention, and 90% of patients ($n = 542$) in the racial-match analyses had available data. Both the WAIP (Wilks's $\lambda = 0.99$, $p < .05$) and WAIT (Wilks's $\lambda = 0.97$, $p = .001$) discriminated between completers and noncompleters. Unlike the alliance data, therapist years of experience did not discriminate between completers and noncompleters (Wilks's $\lambda = 1.00$, ns), and therapist degree was unrelated to treatment retention, $\chi^2(3, N = 571) = 3.32$, ns .

Do Gender and Racial Matching Contribute to the Alliance?

1. Generally speaking, girls rated alliance higher. An independent samples t test revealed that girls rated early alliance higher than boys, $t(2, 356) = 2.56$, $p = .01$. Subsequently, a t test comparing girls with male therapists with girls with female therapists found no difference in alliance, $t(2, 68) = 0.52$, ns , suggesting that girls' report of alliance is unrelated to the gender of their therapist.

2. As hypothesized, patients matched on gender with their therapist reported higher early alliances than those mismatched on gender. More specifically, results of independent samples t

tests revealed that compared with mismatched gender pairs, matched gender pairs reported higher WAI scores on the basis of patient report, $t(2, 356) = 3.06$, $p < .01$, but not on therapist report, $t(2, 349) = 1.82$, ns . These results appear to be largely influenced by boys rating their alliance with female therapists very low and girls reporting especially high alliances with female therapists. In order to test this observation, the specific gender match categories were entered into an analysis of variance, controlling for both therapist experience and degree, and differences were found in both the WAIP, $F(4, 356) = 6.35$, $p < .01$, and WAIT, $F(4, 349) = 3.85$, $p < .05$, samples. Post hoc analyses using Tukey's honestly significant difference (HSD) test produced a significant result for male patient-female therapist compared with female patient-female therapist within the WAIP sample ($p = .01$) and the WAIT sample ($p = .05$; see Table 1). Adolescent girls matched with female therapists produced higher alliance ratings from both patient and therapist perspectives than did male patients with female therapists. No other dyads significantly differed from one another.

3. Given the strong findings for gender matching, we wondered whether this was influenced by family structure. Particularly for males, one might expect that boys living in single-parent homes with only their mothers would develop a stronger alliance with male therapists than boys from intact, two-parent homes. In this sample, of those boys treated by male therapists, 91 came from single-parent homes with only their mothers, and 49 came from two-parent homes. An independent samples t test revealed no significant difference between the groups on patient-rated alliance, $t(2, 140) = 0.62$, ns , suggesting that family structure does not influence this relationship.

4. Male therapists report their alliance with female patients significantly lower than do female patients who report their alliance with male therapists. Simply evaluating the rank order of each matched group (see Table 1) across the two measures yields an interesting finding. Male therapists rating their alliance with female patients produced the lowest mean ($M = 59.48$). However, adolescent girls reporting on their relationship with male therapists produced the second highest mean ($M = 74.59$). Initially one might assume this to be a spurious result given the female patients' general tendency to rate the alliance higher than boys and the patients' general tendency to report the alliance higher than therapists. However, an independent samples t test, controlling for population mean differences in the WAIP and WAIT, yielded a significant finding, $t(2, 22) = 2.38$, $p < .05$. Adolescent girls viewed their alliance with male therapists to be stronger than the male therapists' interpretation of the adolescent girls' perception of the alliance. Because the only difference between the two measures is the respondent, one can conclude that male therapists are less inclined to feel their work with adolescent girls is building as strong a relationship as the girls appear to actually feel.

5. Patients and therapists disagreed about the role of race on the developing alliance. According to patient report, being matched with a therapist of the same race did not contribute to a stronger early alliance, $t(2, 334) = 1.15$, ns (see Table 2). Therapists, however, believed that being matched with a patient of the same race would lead the patient to feel a stronger alliance with them,

$t(2, 327) = 3.59, p < .01$.³ When the specific race categories were entered into an analysis of variance model, the results held with no between-group differences on the WAIP, $F(4, 334) = 1.33, ns$, but significant differences between groups on the WAIT, $F(4, 327) = 9.21, p < .001$. Post hoc analyses using Tukey's HSD revealed that Caucasian therapists rated their alliance with Caucasian patients significantly higher than their alliance with minority patients ($p < .001$). Similarly, minority therapists rated their alliance higher with minority patients compared with Caucasian patients ($p = .01$). Finally, Caucasian therapists reported higher alliances with Caucasian patients than minority therapists reported with Caucasian patients ($p < .001$). Again, the only difference in the measures is the respondent. So, although mental health professionals and those who provide referrals often feel that having their patients matched with someone of the same race may generate a greater collaboration on working toward treatment goals, a belief supported by the WAIT data, after only two therapy sessions, race does not appear to be a contributing factor in the adolescents' view of the alliance.

Do Gender and Racial Matching Contribute to Treatment Retention?

1. A gender match led to greater treatment retention. Dichotomizing the gender match into "same gender" and "different gender" produced a significantly higher rate of completion of two thirds of treatment in the same-gender group, $\chi^2(2, N = 571) = 8.20, p = .01$ (see Table 3). Seventy-seven percent of those matched with a therapist of the same gender completed two thirds of treatment compared with 66% of those matched with a therapist of the other gender.

2. After utilizing the specific gender match categories, we found that adolescent boys are more likely to stay in treatment longer when they have a male therapist, $\chi^2(2, N = 471) = 7.33, p = .01$. Furthermore, female therapists are more likely to retain adolescent girls in treatment than adolescent boys, $\chi^2(2, N = 359) = 4.41, p < .05$.

3. Even though patients did not report differences in early alliance on the basis of being matched with a same-race therapist, race significantly influenced treatment retention. Only 55% of patients matched with different-race therapists completed two thirds of treatment compared with 79% of patients matched with same-race therapists, yielding a significant finding, $\chi^2(2, N = 452) = 34.54, p < .001$.

4. Caucasian therapists treating minority patients had significantly lower retention rates than all other race categories. In the 122 cases in which Caucasian therapists were treating minority patients, only 48% of patients completed two thirds of treatment—a rate slightly lower than chance. Furthermore, 79% of Caucasian patients treated by Caucasian therapists completed at least two thirds of treatment, the largest retention rate of all of the race categories. This retention rate is significantly higher than the 66% of Caucasian patients treated by minority therapists who completed two thirds of their intended treatment, $\chi^2(2, N = 352) = 3.87, p = .05$.

Conclusions and Clinical Implications

Although the concept of matching patients and therapists on gender and race has long been debated in the treatment literature,

relatively little is known about the effects of matched dyads in adolescent psychotherapy. The present study investigated the impact of this match by looking at alliance and treatment retention in a sample treated in a large multisite, clinical trial of adolescents in outpatient substance abuse treatment. In addition, the data set had the unique opportunity to compare patient and therapist ratings of alliance.

From a patient perspective, a gender match with the therapist facilitated alliance development, and gender-matched dyads were more likely to complete two thirds of treatment. For adolescent girls, perhaps having a female role model may help the developing alliance. A second possibility stems from the literature on female adolescent development, suggesting that girls identify themselves through relational connections and, in turn, may place greater value on relationships than boys (Gilligan, 1982). Thus, the potential for interpersonal bonding in therapy may be very appealing to girls. To the extent that this gender characteristic is true, female therapists most likely use similar interpersonal approaches within the therapy setting. Therefore, the match of interpersonal styles may be what is mediating this increase in the alliance score.

For boys, the relationship with female therapists appears different. Adolescent boys rate their alliance with female therapists considerably lower and are more likely to drop out before completing two thirds of treatment than any other gender dyad. Perhaps the interpersonal processes that are engaging for adolescent girls are the very same processes that turn adolescent boys away from female therapists. In addition, gender identification is at its peak during adolescence, and boys may seek affiliation with older male role models. Perhaps feeling close to female therapists seems antithetical to this developmental task, thus prohibiting alliance building in these mismatched dyads. Future research is necessary to test these and other potential explanatory theories, but clearly these data suggest that boys feel better about treatment when working with a male therapist.

An interesting finding is evident when looking across patient and therapist alliance ratings. The greatest discrepancy between patient-rated and therapist-rated alliance existed between adolescent girls matched with male therapists. Male therapists rated the alliance significantly lower than the girls. Maybe this indicates that male therapists believe they cannot connect as easily with adolescent girls, or they have an underlying discomfort when working with adolescent girls. In an early investigation of countertransference, Howard, Orlinsky, and Hill (1969) found that, compared with female therapists, male therapists reported greater personal discomfort when treating female patients. Given the especially high need for relatedness evident in substance-abusing adolescent girls (Blake, Amaro, Schwartz, & Flinchbaugh, 2001), Howard and colleagues' early findings may hold some merit with this population. Simply stated, the need for affiliation may best be expressed within relationships in which one more readily identifies with the other on the basis of common traits or characteristics.

³ Recall that African Americans paired with Latino individuals were viewed as "different race" in the same race–different race breakdown in this analysis. In order to be classified as "same race," both the patient and therapist have to be identified as African American, Caucasian, or Latino. An African American therapist treating a Latino adolescent is considered a mismatch and is therefore classified as "different race."

From a clinical perspective, practitioners must remain cognizant of an adolescent's need, or lack thereof, for relatedness, particularly female therapists working with seemingly resistant adolescent boys. Male therapists cannot underestimate an adolescent girl's need for affiliation. In both cases, identifying patient-therapist commonalities and allowing for appropriate exploration of connection and interpersonal needs may be an essential element of the treatment process.

Racial matching, although encouraged for promoting culturally competent treatment practices (Campbell & Alexander, 2002; Sue et al., 1991), did not influence whether the adolescent reported high alliance. These data challenge the frequent assumption that therapists from similar racial backgrounds as their patients will be more accepted by their patients. Our clinical experience suggests that although same-race matching has an initial appeal to patients, it is the quality of respect and connection felt by the patient that quickly supersedes racial differences. Despite the insignificant difference in patient-rated alliance between matched and mismatched dyads, some topics (e.g., racism) may remain easier to broach with a same-race therapist (Cardemil & Battle, 2003; Knox, Burkard, Johnson, Suzuki, & Ponterotto, 2003).

Racial mismatching did have a negative impact on retention, particularly when minority patients were being treated by Caucasian therapists. Why adolescents in mismatched racial dyads terminate from treatment early is open to debate. Perhaps lack of trust for Caucasians is one explanation for high treatment dropout among minorities, particularly African Americans. When assigned to Caucasian therapists, mistrustful African Americans expect less support from their therapists and, consequently, believe that less positive outcomes will result (Watkins & Terrell, 1988). In addition, therapists may be contributing to poor retention rates. Our data indicate that mismatched therapists rate alliance much lower than do adolescents. This may reflect a more accurate rating of the alliance, therapist discomfort working with patients different from themselves, or merely a difficulty in understanding affiliative cues from these patients. In any case, these perceptions may manifest in observable therapist behavior perceivable by the adolescent, leading to their own discomfort in therapy and early termination. Future research to tease apart these mediational processes is necessary before firm conclusions can be made.

In general, two points can be made. First, this study supports the American Psychological Association's (2003) emphasis on multicultural training. Racial differences between patient and therapist seem to contribute to attrition, particularly for Caucasian therapists who are working with minority patients. Maramba and Hall (2002) argued that with proper training, therapists can deliver culturally competent psychotherapeutic services. Such training needs are well articulated in recent literature (e.g., Cardemil & Battle, 2003; Stuart, 2004; Thompson et al., 2004). In fact, it is possible that the present emphasis on multicultural training contributed to the null findings of patient-reported alliance. Perhaps the study therapists were delivering more culturally competent treatments, leading to higher than expected ratings of alliance by adolescents in mismatched dyads. Similarly, it may have been the therapists' own sensitivity to and awareness of these issues that led to lower therapist-reported alliance ratings in mismatched dyads.

Second, our data also suggest that gender competency may be another important area that deserves a stronger training emphasis. Although racial match mattered more than gender match for ther-

apist views of alliance, patient-rated alliance resulted in the opposite effect. Same-gender dyads reported higher alliance ratings, but same-race dyads did not. If developing a strong therapeutic relationship is truly essential to the curative process (Greenson, 1967; Sullivan, 1940), then our data suggest that recognizing and working with patient-therapist gender differences may be as, or more, valuable than tackling racial differences. Although racism and beliefs concerning race transcend age groups (Katz, 2003), gender identification may be more central to the developmental needs of adolescents. Adolescence is a period during which gender role prescriptions intensify, and gendered messages are often conflicting and confusing. For these reasons, identification with individuals who can help navigate through this sensitive life stage is clearly beneficial in the therapeutic context. Given the primacy of gender in identity development, and the central role of intimacy in that process, particularly for girls (Gilligan, 1982), it is not surprising that female-female gender-matched dyads in this population brought about the greatest successes with respect to alliance and retention. In fact, substance abuse prevention programs are now being developed that target gender differences (see Blake et al., 2001, for a review). Recognizing the significant role gender plays in the development of adolescents is necessary if practitioners are to effectively engage teens in the treatment process and, ultimately, facilitate positive therapeutic change.

This study is limited by the broad nature of the data collected. The moderating effects of national origin, immigration history, and acculturation are obscured by the broad panethnic labels used to describe the patients (Fisher et al., 2002). In contrast to matching on race, Sue and colleagues (1991) outlined important elements of cultural match, including shared language, understanding the patient's background, and openness to modifying treatment accordingly. In addition to the racial and ethnic limitations of the data collection, sample size precluded any in-depth analyses of ethnic or cultural differences within the population. Sue and Zane (1987) argued that cultural match is more proximal to therapy, thus making it a better predictor of treatment outcome than ethnic match. In this study, the three minority therapists had significantly better retention rates with their mismatched patients than Caucasian therapists had with their mismatched patients. It is possible that their shared experience of being a minority allowed them greater sensitivity to issues surrounding racial differences, thus providing them with invaluable tools when working with their racially mismatched patients.

Whenever data are collected from multiple informants, the "truth" of the data are subject to interpretation. Reporting bias may also influence the value of these data. Although adolescents were promised confidentiality regarding alliance data, they may still give socially desirable responses. In particular, the adjudicated adolescents (62% of the sample), who were partially coerced into treatment, may feel compelled to appear cooperative with treatment. For some of these youth, "successful" completion of a substance abuse treatment program was a requirement of probation. Alternatively, therapists have less motivation to present socially desirable responses. They were also more familiar with the construct being measured and the challenges of engaging adolescents in treatment. However, one could argue that their report on alliance is biased by the stereotyped bravado and/or indifference that characterize many adolescents presenting for drug treatment. The true nature of alliance likely lies somewhere between the two

reports. Exactly where is a matter of individual differences and a topic for future research.

One may question the generalizability of these findings. For instance, do the adolescents in the CYT study represent patients seen in the larger substance use treatment system? On this score, the CYT sample has been compared with patients in the national treatment system and found to be very similar, including a 50% referral rate from juvenile justice (Dennis et al., 2004). One may also wonder whether these findings generalize to adolescents seeking services in the general mental health population. Unfortunately, this question cannot be fully answered. However, the high rate of co-occurring mental health problems in this sample provides further evidence that the distinction between these two types of problems and patient populations is rapidly disintegrating (U. S. Department of Health and Human Services, 2002). Finally, one has to consider the generalizability of the therapist sample. Fortunately, 14 therapists were used in the CYT study, consisting of paraprofessionals, master's-level clinicians, social workers, and psychologists. Although this may not fully represent all therapists, it provides an adequate representation for this initial study and is characteristic of the many individuals who treat adolescents in community agencies, hospital settings, and independent practice.

Overall, this study explored the role of gender and racial matching within adolescent populations. Understanding our similarities and differences with patients may help strengthen the alliance and retain patients in treatment. Alliance and retention are necessary but not sufficient for successful treatment. However, awareness of the processes that contribute to these factors may enhance therapists' ability to effectively deliver treatment to this challenging population.

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